



Estimating US savings on outpatient prescription pharmaceuticals from international reference pricing

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The exorbitant cost of prescription pharmaceuticals is a critical and long-standing concern facing the United States (US). To curb rising drug costs, international reference pricing, aligning drug prices with those in other high-income countries, has emerged as a prominent policy option. We evaluated the potential national savings from such a strategy in the United States through a comprehensive analysis of utilization patterns and global price differentials for over 8,000 drugs. We project an annual national saving of \$184 billion if outpatient prescription pharmaceuticals were priced comparably to those in Canada, France, Germany, Japan, and the United Kingdom. Private insurers, Medicare, and Medicaid would realize savings of 51%, 62%, and 35%, respectively. Pharmaceutical expenditures for common chronic illnesses would be substantially reduced, with costs declining by over 75% for diabetes, atrial fibrillation, and heart failure. Our findings emphasize the substantial fiscal and equity gains achievable through international reference pricing, demonstrating its potential to mitigate financial burdens borne by both healthcare systems and individual patients.

international reference pricing | pharmaceutical expenditure | health policy | healthcare savings

The escalating cost of healthcare is a growing concern for both policymakers and the public in the United States (US). In 2023, healthcare costs in the United States exceeded \$4.8 trillion, representing 17.6% of the gross domestic product, with projections indicating a rise to 19.7% by 2028 (1, 2). Inflated drug prices are a major driver of this unsustainable healthcare spending. Compared to many other high-income countries, the United States often pays two to four times higher prices for the same medications (3).

High drug prices impose considerable financial burdens on US patients, jeopardizing their ability to access necessary treatments (4–6). A recent Kaiser Family Foundation poll found that 28% of US adults struggle to afford their medications, and more than 30% report cost-related nonadherence (7). While most Americans have health insurance and may only pay a portion of the list price, these soaring drug costs are ultimately reflected in higher premiums, deductibles, and copays. For the estimated 25.3 million uninsured Americans (8), the burden is far heavier, as they lack the negotiating leverage of insurers and typically pay full price out of pocket.

Rising pharmaceutical costs also strain the budgets of public insurance programs, such as Medicare, which serves seniors, and Medicaid, which serves low-income individuals and those with disabilities (9, 10). Drug costs comprise a growing share of public insurance spending over the past decade, a trend expected to continue (11, 12). For Medicaid, drugs costing over \$1,000 per claim climbed from 41% of its gross spending on outpatient prescription drugs in 2016 to 62% in 2023 (13). Medicare Part D observed a similar rise, from 33 to 59% (13). This significant shift threatens both equitable access to life-saving medications and the financial sustainability of these programs.

Improving prescription drug accessibility and affordability has long been a critical policy issue in the United States. The Inflation Reduction Act of 2022 marked a turning point (14), for the first time authorizing Medicare to negotiate prices for select high-cost drugs and penalize manufacturers for unwarranted price hikes. Complementing these efforts, international reference pricing is a mechanism that calls for pegging US drug prices to those in countries with similar economic status. International referencing pricing has been a bipartisan theme in legislative proposals and executive action, including the Elijah E. Cummings Lower Drug Costs Now Act of 2019 (15), the Prescription Drug Price Relief Acts of 2021 and 2025 (16–19), and the Delivering Most-Favored-Nation Prescription Drug Pricing to American Patients presidential executive order of 2025 (20).

Effective negotiation strategies and policy reforms depend on a comprehensive analysis of prescription drug utilization and international price disparities. While studies have consistently highlighted stark differences in drug prices between the United States and

Significance

Prescription pharmaceutical costs in the United States (US) are among highest in the world, placing substantial financial strain on patients, public insurance programs, and private payers. International reference pricing, pegging US drug prices to those in other high-income countries, has gained bipartisan support as a potential solution. This study offers a comprehensive analysis of the financial impact of such a pricing strategy, providing timely and valuable implications to inform ongoing drug pricing reform efforts and advance equitable access to essential medications.

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